جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مريان نعيم حكيم بلامون
المحافظة : القاهره **الحي / المدينة :** الدراسه **العنوان :** ///
رقم البطاقة : 28309300108144 **رقم التليفون :** 01221080735 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 30-09-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مريان نعيم حكيم بلامون
National ID: 28309300108144

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





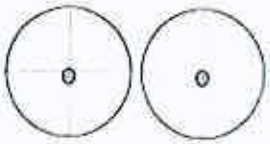
National ID · Front & Back
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Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

مريان نعيم هكرم بلامون	Sex:	3- Medical number: Female	83161171	Old Med :	3323-2023
nber: 28309300108144		Patient Age : 40 Y			



Family history	+ve	Phone	01221080735	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	23/04/2024
Co morbidity	+ve	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
LT UIQ pathological single mass with ipsilateral indeterminate L.N. BIRADS6

Mets Workup :
LT Breast malignant mass associated with multiple enlarged bilateral axillary and internal mmo nary L.N. likely metastasis
NO CT evidence of pulmonary or hepatic

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	6/8	0		35

T	N	M
2	3b	0

Sonomammography

PATIENT HISTORY
P.T CLIP INSERTED NACT 3 AC , 3 TAXOTAIR LAST ONE 21/2/2024
WITH GOOD RESPONSE
O/E NO AXILLARY OR SUPRACLAVICULAR LN
BSM CR OF THE AXILLARY LN WITH SD OF THE LEFT BREAST MASS
FOR LEFT WLE(WIRE)& SLNB:idc gii
Partial pathological response
celluarity 8&

MDT OUTCOME & PLAN

LHRH and AI ,,PORT , zometa 16 months

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: مريان نعيم حكيم **MR. :** 5543	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 42-year-old individual with invasive ductal carcinoma, grade II, clinical stage T2 N3b. Neoadjuvant chemotherapy was administered consisting of three cycles of AC followed by three cycles of taxane (TXT). After completion of neoadjuvant therapy, the patient underwent left wide local excision and sentinel lymph node biopsy, which demonstrated ypT2 ypN0. Post-operative treatment included adjuvant endocrine therapy with an aromatase inhibitor and Zoladex initiated in April 2024 and continued to the present, along with supplemental therapy. The patient received three months of zoledronic acid (Zometa) after laboratory evaluation; the most recent bone scan in April 2026 was pending, and the patient is currently disease-free.

The patient presented for administration of Zoladex, Arimidex, and supplemental therapy for three months and reported new neck pain radiating to both upper limbs. A cervical MRI was ordered for further evaluation. The plan is to review MRI findings and manage the neck pain accordingly.

تحريرا في ,

التاريخ :	**18-09-2026**	**الوقت :**	

الطبيب المعالج : **Reham Ahmed Fathy Ibrahim**

يعتمد ,

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Merlan Naeem Hakim Blamon	Date : 21/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 3323/2023	Pat. ID: 47082

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Known case of left breast cancer on therapy with clip

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left LIQ spiculated medium dense lesion with clip seen within, measuring about 50 x33mm (in CC view).
- No Right spiculated dense masses or suspicious microcalcifications

Complementary ultrasound :

- Left 9 O'clock still noted irregular hypoechoic solid mass , measured about 15x10mm >>> compared to previous study dated at 7-2023 measuring about 17x12mm
- No right cystic or solid mass lesions.
- Normal right skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Nonspecific right axillary LN.
- No left pathological axillary lymph nodes.
- Free supra and infra clavicular lymph nodes.

Opinion:

- Left UIQ pathologically proven carcinoma showing no radiological response (BIRADS 6).
- Good response of left pathological axillary lymph nodes.
- Right breast benign finding (BIRADS 2).

Signature Dr. Nehal Hamdy

21/02/2024 2:45 PM

[[DOCTYPE:MAMMOGRAM]]

Patient Name	[illegible]	Request No	[illegible]
Patient No	[illegible]	Request Date	[illegible]
File No	[illegible]	Verification Date	[illegible]
Sex	Female	Financial Category	[illegible]
Age	45 Y	Department	Out Patient
Referring Doc.	-	Registered by	[illegible]

Pathology Report

Clinical Data: Right axillary lymph node

Nature of Specimen: Tru-cut biopsy, collected at [illegible] 12:30 pm

Gross Examination
- Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination
-reveals preserved normal nodal architecture. The subcapsular sinuses are patent. The lymph follicles are hyperactive and have prominent germinal centers. There is no evidence of specific granuloma or neoplasia in sections examined.

Diagnosis
Right axillary lymph node, Tru-cut biopsy
Reactive lymph node, No malignancy

Signature
Prof. Dr. Lubna Omar.
Dr. Heba Hassan.

Verified by :
DR.Menna Nabil

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – ميت القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mist El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

قسم الاشعة التشخيصية

بـ ـيـة

PATIENT NAME: **MARYAN NAEEM HAKIM Balamon** GENDER: **Female** AGE: **39 y**	PATIENT ID #: **10123231507** ORDER #: **2483390,2483391** DATE OF SERVICE: **10-Sep-2023 11:03 AM**

Report: - all studies

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- left breast malignant mass

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR b).
- Left UIQ spiculated dense mass showing intense heterogenous mass enhancement measuring about 4.5 x3.5 cm (including the spicules)and located at about 4 cm from the nipple. Showed by US deep to the skin by about 1 cm.
- No right spiculated mass lesions or microcalcific clusters.
- Contrast Enhanced Spectral Mammography showed mild back ground parenchymal enhancement with no significant enhancing lesions in the right side.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Bilateral axillary (level I,II) indeterminate enlarged lymph nodes with diffuse cortical thickening , the largest on the left side measures about 22.5 x11.5 mm with 7 mm cortical thickness and on the right side measures about 13.5 x8 mm and about 5 mm cortical thickness .

OPINION:

- Left UIQ pathological enhancing single mass with bilateral indeterminate lymph nodes for FNAC from ipsilateral axillary lymph nodes (BIRADS 6).
- No right breast pathologically enhancing lesions (BIRADS 2).

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Merian Naeem Hakim Blamon	Date : 03/03/2025
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID: 3323/2023	Pat. ID: 47082

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)

Left breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post left BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: MARYAN NAEEM HAKIM Balamon GENDER: Female AGE: 39 Y	PATIENT ID #: [illegible] ORDER #: [illegible] DATE OF SERVICE: 10-Sep-2023 11:03 AM

Report: - all studies

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- left breast malignant mass

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR b).
- Left UIQ spiculated dense mass showing intense heterogenous mass enhancement measuring about 4.5 x3.5 cm (including the spicules)and located at about 4 cm from the nipple. Showed by US deep to the skin by about 1 cm.
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OPINION:

- Left UIQ pathological enhancing single mass with bilateral Indeterminate lymph nodes for FNAC from Ipsilateral axillary lymph nodes (BIRADS 6).
- No right breast pathologically enhancing lesions (BIRADS 2).

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذه / مريان نعيم حكيم بلامون	Request No.	: 13001655
Patient No.	: 28309300108144	Request Date	: 02/10/2023
Path No.	: 3356/23	Verification Date	: 15/10/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right axillary lymph node.

Nature of Specimen: Tru-cut biopsy, collected at 1/10/2023, 12:30 pm.

Gross Examination:
Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:
Reveals preserved normal nodal architecture. The subcapsular sinuses are patent. The lymph follicles are hyperactive and have prominent germinal centers. There is no evidence of specific granuloma or neoplasia in sections examined.

Diagnosis:
Right axillary lymph node, Tru-cut biopsy:
Reactive lymph node, No malignancy.

Signature
Prof. Dr. Lubna Omar.
Dr. Heba Hassan.

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: **MARYAN NAEEM HAKIM Balamon** GENDER: **Female** AGE: **39 y**	PATIENT ID #: **10123231507** ORDER #: **2475789,2475790** DATE OF SERVICE: **05-Sep-2023 10:59 AM**

Report: - all studies

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

1-CHEST

- Left breast UIQ ill defined heterogenous enhancing soft tissue mass lesion is with internal cystic changes, measuring about 19x27 mm. LOQ subcentimetric well defined nodule measuring about 9 mm.
- Multiple pathologically enlarged left axillary (levels 1, 2 and 3) and right axillary (level 1 and 2) lymph nodes, the largest measures about 1 ix 14 mm on left and 12x 11mm.
- Few enlarged left supraclavicular/ lower deep cervical lymph node, reaching about 9x 14 mm.
- Few subcentimetric left internal mammary lymph nodes, the largest measures about 9x 8 mm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No pathologically enlarged hilar and mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is enlarged in size with homogenous CT texture. No definite hepatic focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen (small splenule is seen related to its anterior pole), adrenals, aorta, I.V.C and the other pelvic organs.

CT Scan
[[DOCTYPE:CT_SCAN]]



جمهورية مصر العربية
بطاقة تحقيق الشخصية

مريان
نعيم حكيم بلامون
٩ش عبدالعظيم ابراهيم
حدائق القبة - القاهره

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J00819553

DMS

ارامبيكس + زوجته